

EB-2 NIW — Dhanasar Three-Prong Analysis

Manasa Jampani — Ardia Health Labs

Legal Standard: Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016)

Petition Type: Form I-140, Preference Category EB-2, National Interest Waiver

Petitioner: Manasa Jampani, Co-Founder & CEO, Ardia Health Labs

Overview

The Dhanasar framework establishes three prongs for National Interest Waiver eligibility. All three prongs are satisfied for Manasa Jampani:

1. **Prong 1 — Substantial Merit and National Importance:** The petitioner's proposed endeavor has substantial merit and national importance  STRONG
 2. **Prong 2 — Well Positioned to Advance the Endeavor:** The petitioner is well positioned to advance the proposed endeavor  STRONG
 3. **Prong 3 — On Balance, It Would Be Beneficial to the United States to Waive the Labor Certification Requirement:** It would be beneficial to the US to waive labor certification  STRONG
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Prong 1: Substantial Merit and National Importance

The Endeavor

Manasa Jampani's proposed endeavor is the co-development and commercialization of AI-native revenue cycle management infrastructure for independent clinical laboratories in the United States, specifically addressing claim denial recovery in the molecular diagnostics, pharmacogenomics, and next-generation sequencing segments.

Substantial Merit

Economic Merit

The independent clinical laboratory billing domain represents an estimated \$10-12 billion in annual preventable revenue loss attributable to claim denials that either go unappealed or fail due to inadequate documentation. This figure derives from two sources:

- XiFin 2024 Payor Denial Impact Report (20+ million claims): Molecular diagnostics denial rate of 35.3% — the highest across all healthcare specialties
- HFMA/LigoLab 2024 Laboratory Revenue Recovery Report: 65% of denied claims never appealed, despite 50–80.7% appeal win rates

The economic value of reducing this denial rate and closing the appeal gap is directly quantifiable: recovering even 10% of the estimated \$10–12 billion annual loss would represent \$1–1.2 billion in additional revenue annually to independent laboratories — laboratories that employ thousands of healthcare workers and provide essential diagnostic infrastructure.

Clinical Merit

Independent clinical laboratories are the diagnostic infrastructure for: - Rural and underserved communities served by Critical Access Hospitals and independent clinics - Addiction medicine practices relying on clinical toxicology panels for substance use disorder treatment monitoring (a national public health priority) - Specialty oncology practices relying on NGS panels for precision cancer treatment selection - Pharmacogenomics-guided prescribing, which offers the potential to reduce adverse drug events from medications with FDA-recognized pharmacogenomic biomarkers

When independent laboratories lose revenue to preventable claim denials and close or consolidate, these communities lose access to essential diagnostic services that are not replaced by hospital-based laboratories serving primarily inpatient and affiliated outpatient populations.

Technical Merit

The AI architecture being developed — the DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture, invented by Ram Vadlamudi — addresses a genuine technical problem that existing platforms cannot solve. The MolDX program complexity, DEX Z-code compliance requirements, and CPT 81400–81479 coding requirements for genomic sequencing represent a technical domain with no existing commercial AI solution. Manasa Jampani’s contribution is the payer-side operational intelligence and business strategy that shapes the architecture’s countermeasure design.

National Importance

Scale of Impact

- \$10–12 billion annual preventable revenue loss — a quantifiable economic burden on the US healthcare system
- 7,000+ CLIA-certified independent laboratories at financial risk

- More than 50% of independent laboratory directors considering consolidation or sale if PAMA 2027 reimbursement cuts take effect (ACLA 2024)
- PAMA 2027: Up to 45% cumulative Medicare reimbursement cut for clinical laboratory tests by 2029

Healthcare Access Equity

The 2.76× higher denial odds for independent vs. hospital-based laboratories documented in the JAMA Network Open 2025 study (n=29,919) represents a structural healthcare access inequity. Independent laboratories serve populations that hospital-based laboratories often do not reach. AI-driven claim recovery that reduces this denial disparity addresses a national healthcare equity concern.

Healthcare Infrastructure Preservation

The independent laboratory sector provides 32% of US diagnostic test volume. Systemic financial pressure from PAMA cuts and preventable denial rates threatens to accelerate the consolidation of this sector into large national reference laboratory networks, reducing the geographic distribution of diagnostic services and increasing turnaround times for community-serving tests.

AI Governance Leadership

Ardia Health Labs built its platform in native compliance with Texas SB 1188 (effective September 1, 2025) and Texas TRAIGA (HB 149, effective January 1, 2026) — the first healthcare-specific AI accountability law and the third state comprehensive AI governance law in the United States, respectively. The development of responsible AI governance practices in the healthcare administrative domain has national significance for the US AI regulatory framework.

Prong 2: Well Positioned to Advance the Endeavor

Manasa Jampani's Unique Qualifications

A. Payer-Side Insider Knowledge — The Irreplaceable Asset

Manasa Jampani has 5+ years of healthcare IT experience at Interwell Health and United HealthGroup — organizations that operate on the payer and payer-adjacent side of the healthcare revenue cycle. This experience provided direct operational visibility into:

- How payer clinical operations teams configure automated claim denial systems

- The documentation requirements and coverage criteria that trigger or prevent automated denials
- Prior authorization AI decision engine configuration and clinical criteria weighting
- Payer claim edit libraries and code combination validation rules

This knowledge is:

1. **Not available from public sources:** Payer claim edit libraries, coverage determination matrices, and prior authorization AI scoring criteria are proprietary and not disclosed in public payer documentation
2. **Not available to outside consultants:** Revenue cycle management firms, clearinghouses, and RCM technology vendors observe payer behavior from outside and reverse-engineer denial patterns; they do not have access to the operational configuration decisions that produce them
3. **Not replicable from claims data:** Published academic analyses of denial outcomes (e.g., JAMA Network Open 2025) document what payers deny and when — they cannot characterize the operational logic of why, which requires inside knowledge

Manasa Jampani's specific roles at United HealthGroup and Interwell Health placed her in proximity to payer clinical operations functions. The competitive advantage this creates for Ardia Health Labs — and specifically for the DGP architecture's countermeasure design — is categorical, not marginal.

B. ACO Operations Experience

Manasa Jampani's experience in ACO (Accountable Care Organization) operations at Interwell Health provides strategic intelligence for the DGP AcoIQ™ product — a module designed to optimize ACO shared savings performance for independent laboratories participating in value-based care arrangements. This domain combines her renal care management expertise with her payer-strategy background.

C. CEO and GTM Leadership

As Co-Founder and CEO, Manasa is responsible for investor relations, go-to-market strategy, and clinical operations partnerships. Her prior roles at United HealthGroup and Interwell Health gave her executive-level relationships within the payer ecosystem — relationships that are directly relevant to establishing Ardia's commercial partnerships and pilot program negotiations.

D. Platform Completion Status

As of March 2026, the Ardia Health Labs platform architecture is 100% complete. The company is actively pursuing its first pilot program with a DFW-area independent laboratory. Manasa Jampani is the operating executive responsible for converting this technical completion into commercial deployment — demonstrating that her endeavor is not hypothetical but actively advancing.

E. Publication Record — Building

Manasa Jampani is a co-author on Papers 2 and 3 (the independent laboratory revenue crisis systematic review and the Texas AI governance paper) and the sole author of Paper 4 (“Inside the Payer AI Black Box”). Paper 4 is uniquely hers: no other researcher can write this paper with the same vantage point. Its targeted submission to NEJM AI — the leading clinical AI journal — directly establishes her expert standing in this domain.

Prong 3: Beneficial to the United States to Waive Labor Certification

No Comparable Domestic Expertise Available

The labor certification process exists to protect US workers from displacement by foreign workers filling roles that qualified US workers are available to fill. Manasa Jampani’s role cannot be filled by a US worker without her specific combination of:

- Direct payer-side clinical operations experience at United HealthGroup and Interwell Health (the specific organizations whose operational knowledge informs the DGP architecture’s countermeasure design)
- Co-founder status and ownership stake in Ardia Health Labs (not a role that can be hired through PERM recruitment)

The labor certification process cannot identify a US worker who has simultaneously: (a) Manasa’s specific payer clinical operations experience at the specific organizations that shaped her insight, AND (b) the entrepreneurial relationship to the company that makes her the appropriate person to advance this endeavor.

Urgency of the National Need

PAMA 2027 creates a time-sensitive national need. If the reimbursement cuts take effect as projected, the consolidation pressure on independent laboratories will accelerate in 2027–2029. AI-driven claim recovery technology that addresses the denial burden before these cuts take effect has maximum national impact during this window. Delay in Manasa Jampani’s continued contribution to this work — through a multi-year PERM

process — would directly reduce the impact of this work on the national need.

No Comparable Platform Exists

No existing commercially available AI platform addresses the specific combination of: - MolDX/LCD/NCD/DEX Z-code compliance for molecular diagnostic billing - Payer AI countermeasure design informed by payer-side operational intelligence - ACO operations optimization for independent laboratory value-based care participation

Ardia Health Labs represents the only platform pursuing this specific combination. Manasa Jampani's continued involvement is essential to its success, and her success serves the national interest.

Preserving Healthcare Infrastructure

Every independent laboratory that survives the PAMA 2027 cuts because it successfully recovered denied claims represents a diagnostic facility that continues serving its community. Manasa Jampani's work at Ardia Health Labs has direct implications for laboratory accessibility in rural, addiction medicine, and specialty healthcare communities — populations that depend on independent laboratories precisely because they lack hospital-system access.

Supporting Evidence Package

Document	Prong	Evidence Type
Ardia Health White Paper (March 2026)	All 3	Technical architecture + market analysis
Pitch Deck v2 (2026)	P1, P3	Market size, problem statement, founder bios
XiFin 2024 Denial Impact Report	P1	Third-party market evidence (35.3% denial rate)
JAMA Network Open 2025 (Georgetown)	P1	Peer-reviewed academic evidence (2.76× odds ratio)
ACLA 2024 Survey	P1, P3	Industry evidence (50%+ labs considering exit)
HFMA/LigoLab 2024 Report	P1	Industry evidence (65% appeal gap)
Texas SB 1188 text	P1, P3	Regulatory evidence (national AI governance significance)
Texas TRAIGA text	P1, P3	Regulatory evidence
Paper 4 (Payer AI	P2	Expert authority evidence

Document	Prong	Evidence Type
Black Box)		(Manasa's unique paper)
Expert witness letters	All 3	Third-party expert corroboration
LinkedIn profile / employer verification	P2	Employment history documentation
ECFMG / United HealthGroup / Interwell employment records	P2	Credential verification